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Anat Path Reports

* Final Report *

Document Type: Anat Path Reports
Document Date:
Document Status: Modified
Document Title: Histology
Performed By:
Verified By:
Encounter info:

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APRPT (Verified)

Patient Name:
MRN:
Location:
Client:
Submitting Phys:

DOB:
Gender: M

Acc #:
Collected:
Received:
Reported:

Final Surgical Pathology Report
*** ADDENDUM PRESENT ***

Addendum Diagnosis
The diagnosis is not changed.

Addendum Comment
Elastic stains (X2) were evaluated for visceral pleural invasion.

Assisted by:

Final Pathologic Diagnosis
A. LYMPH NODES, RIGHT LEVEL 9, EXCISION.

Printed by:
Printed on:

- FIVE LYMPH NODES NEGATIVE FOR METASTATIC CARCINOMA (0/5)

B. LYMPH NODES, LEVEL 7, EXCISION
- THIRTEEN LYMPH NODES NEGATIVE FOR METASTATIC CARCINOMA (0/13)

C. LYMPH NODES, RIGHT LEVEL 14, EXCISION
- SEVEN LYMPH NODES NEGATIVE FOR METASTATIC CARCINOMA (0/7)

D. LYMPH NODE, LEVEL 11, EXCISION
- NINE LYMPH NODES NEGATIVE FOR METASTATIC CARCINOMA (0/9)

E. LUNG, RIGHT LOWER LOBE, LOBECTOMY, E1FS & E2FS
- ADENOCARCINOMA, MODERATELY DIFFERENTIATED (3.5 X 3.1 X 2.1 CM).
- TUMOR INVADES INTO VISCERAL PLEURA.
- LYMPH0VASCULAR INVASION IDENTIFIED.
- BRONCHIAL AND VASCULAR MARGINS NEGATIVE.
- ONE OF THREE PERIBRONCHIAL LYMPH NODES POSITIVE FOR METASTATIC CARCINOMA (1/3).
- SEE SYNOPTIC REPORT.

Addendum / Signed Out

Synoptic Worksheet

E. Right lower lobe:

Specimen:	Lobe(s) of lung: Right lower
Procedure:	Lobectomy
Specimen Integrity:	Intact
Specimen Laterality:	Right
Tumor Site:	Lower lobe
Tumor Focality:	Unifocal
Histologic Type:	Adenocarcinoma
Histologic Grade:	G2: Moderately differentiated
Tumor Size:	Greatest dimension: 3.5 cm Additional dimension: 3.1 cm Additional dimension: 2.1 cm
Visceral Pleura Invasion:	Present
Tumor Extension:	Not applicable
Bronchial Margin:	Uninvolved by invasive carcinoma

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	Involvement by squamous cell carcinoma in situ (CIS) not applicable
Vascular Margin:	Uninvolved by invasive carcinoma
Parenchymal Margin:	Not applicable
Parietal Pleural Margin:	Not applicable
Chest Wall Margin:	Not applicable
Other Attached Tissue Margin:	Not applicable
All Margins Uninvolved By Invasive Carcinoma:	
	Distance of invasive carcinoma from closest margin: 23 mm
	Margin closest to invasive carcinoma:: Bronchial
Treatment Effect:	Not applicable
Lymph-Vascular Invasion:	Present
Extranodal Extension:	Not identified
TNM Descriptors:	Not applicable
Primary Tumor (pT):	pT2a: Tumor greater than 3 cm, but 5 cm or less in greatest dimension surrounded by lung or visceral pleura, without bronchoscopic evidence of invasion more proximal than the lobar bronchus (ie, not in the main bronchus); or Tumor 5 cm or less in greatest dimension with any of the following features of extent: involves main bronchus, 2 cm or more distal to the carina; invades the visceral pleura; associated with atelectasis or obstructive pneumonitis that extends to the hilar region but does not involve th the entire lung
Regional Lymph Nodes (pN):	pN1: Metastasis in ipsilateral peribronchial and/or ipsilateral hilar lymph nodes, and intrapulmonary nodes, including involvement by direct extension
	Nodes examined: 37
	Nodes involved: 1
Distant Metastasis (pM):	Not applicable

Clinical History

Lung cancer, right lower lobe. Right VATS and lobectomy. History of prostate cancer. History of tobacco use, one pack a day for ten years, quit in [redacted]

Specimen(s) Received

A: Right level 9 lymph nodes
B: Level 7 lymph nodes
C: Right level 14 lymph nodes
D: Level 11 lymph node
E: Right lower lobe

Gross Description

Specimen A is received in formalin, labeled with the patient's name and medical record number and as "right level #9 lymph node." It consists of multiple fragments of fibroadipose tissue measuring 3.0 x 1.5 x 1.3 cm in aggregate. Within the tissue, multiple lymph nodes are found, the largest of which measures 3.0 x 1.0 x 0.7 cm. Lymph nodes are submitted as stated below.

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Specimen B is received in formalin, labeled with the patient's name and medical record number and as "level #7 lymph node." It consists of multiple fragments of fibroadipose tissue measuring 2.5 x 1.0 x 0.7 cm in aggregate. The specimen is submitted entirely in cassettes "B1" and "B2."

Specimen C is received in formalin, labeled with the patient's name and medical record number and as "right level #14 lymph node." It consists of multiple fragments of fibroadipose tissue measuring 2.5 x 2.0 x 1.0 cm in aggregate. Within the tissue, multiple lymph nodes are found, the largest of which measures 1.7 x 0.9 x 0.6 cm. The lymph nodes are submitted as stated below.

Specimen D is received in formalin, labeled with the patient's name and medical record number and as "right level #11 lymph node." It consists of multiple fragments, 2.0 x 1.5 x 0.5 cm in aggregate. Multiple lymph nodes are identified within the tissue, the largest of which measures 1.0 x 0.7 x 0.5 cm. The lymph nodes are submitted entirely in cassettes 'D1" and "D2." Dictated by

Specimen E is received fresh, labeled with the patient's name, medical record number and as "right lower lobe." The specimen consists of a lobectomy specimen which measures 20.0 x 11.0 x 5.5 cm and weighs 289.5 grams. There is a firm, white, gritty mass that measures 3.5 x 3.1 x 2.1 cm. The mass puckers the overlying pleura. The mass is 2.3 from the bronchial margin. The mass does not invade the vasculature or bronchus. The remainder of the lung is unremarkable. The puckered pleura is inked blue. The bronchial margin is submitted for frozen section diagnosis and the contents of the cryoblock are transferred to cassette "E1FS." A representative of the mass is submitted for frozen section diagnosis and the contents of the cryoblock are transferred to cassette "E2FS." Sections are submitted as stated below.

A1: Large lymph node, bisected
A2: Multiple possible lymph nodes
C1: Three lymph nodes, whole
C2: Multiple possible lymph nodes
E3: Vascular margins
E4,E5: Tumor in relation to pleura (closest area), bisected
E6: Representative section of tumor in relation to bronchus
E7: Tumor in relation to pleura
E8: Section of uninvolved lung
E9: One possible peribronchial lymph node
E10: One possible peribronchial lymph node
E11: One possible peribronchial lymph node
E12: One possible peribronchial lymph node

Intraoperative Consult Diagnosis

E1FS: RIGHT LOWER LOBE (FROZEN SECTION): BRONCHIAL MARGIN WITH RARE CRUSHED, POORLY PRESERVED CELLS IN PERIBRONCHIAL SOFT TISSUE; DEFER TO PERMANENTS.
Frozen section results were communicated to the surgical team and were repeated back by

E2FS: RIGHT LOWER LOBE (FROZEN SECTION): POORLY DIFFERENTIATED ADENOCARCINOMA.
Frozen section results were communicated to the surgical team and were repeated back by

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Pathologist:

Microscopic Description
Microscopic examination performed.